

A NEW GENERATION OF HEALTHCARE INTELLIGENCE

Runda Health Clinical AI

Turning governed medical data into practical AI support before, during and after care.

27
years in medical data

4,000+
healthcare institutions served

40+
clinical AI agents deployed

01 / RUNDA HEALTH

Medical data experience, translated into clinical AI.

Runda combines medical data governance, clinical knowledge and workflow engineering to build AI that can operate inside real hospital environments.

The platform is designed around traceability and clinician control. Outputs remain connected to their source data, governing knowledge and review history.

300+

AI and smart-hospital sites

10+ years

of medical data governance practice

**1
foundation**

supporting a growing portfolio of clinical agents

CLINICAL DATA X / CDX

Not another isolated application. A governed intelligence layer between clinical data and every AI agent.

EXISTING HOSPITAL SYSTEMS REMAIN THE RECORD OF CARE. CDX TAKES APPROVED DATA, TURNS IT INTO USABLE CLINICAL CONTEXT, AND KEEPS EVERY CONCLUSION CONNECTED TO ITS SOURCE.

01**Collect**

Notes, labs, medications and reports from approved sources

02**Normalize**

Terms, units, dates and local abbreviations become comparable facts

03**Connect**

Facts link to longitudinal context, knowledge and governing rules

04**Serve**

Agents receive controlled context and return a traceable result

ONE TRACEABLE EXAMPLE

Clinical note + lab result → normalized clinical fact → Agent reasoning → clinician review

One governed patient context can support CDSA, documentation, quality, HeartEngine and the rest of Runda's 40+ clinical AI portfolio.

02 / REPRESENTATIVE PORTFOLIO

Seven selected agents from a portfolio of more than 40.

These seven examples show how the wider portfolio supports clinical documentation, decision support, specialty care and patient engagement across the care journey.

BEFORE CARE

Prepare the encounter before clinical time begins.

BEFORE CARE / PATIENT INTAKE**01**

AI Pre-Consultation

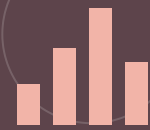


Collects symptoms, history, medications and allergies before the visit, then prepares a structured summary for clinician review.

BEFORE CARE / SCREENING

02

Breast Health



Supports risk assessment, screening guidance and continuity across follow-up care in a patient-friendly workflow.

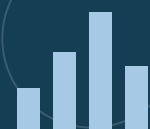
DURING CARE

Bring evidence, documentation and risk analysis into the clinical workflow.

DURING CARE / CLINICAL REASONING

03

CDSA



Turns longitudinal history, clinical knowledge and evidence into a traceable analysis for clinician review.

DURING CARE / DOCUMENTATION

04

Medical Record Generation

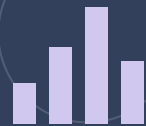


Uses encounter data to prepare an editable clinical draft. The clinician reviews, changes and signs the final record.

DURING CARE / DOCUMENTATION QUALITY

05

Medical Record Quality

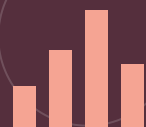


Checks completeness, consistency and quality requirements before the medical record is finalized.

DURING CARE / SPECIALTY INTELLIGENCE

06

HeartEngine



Connects ECG, biomarkers, vital signs and history to a traceable cardiovascular risk analysis.

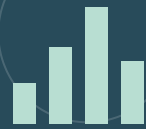
AFTER CARE

Extend governed support beyond discharge.

AFTER CARE / CONTINUOUS SUPPORT

07

AI Health Assistant



Supports education, follow-up and symptom collection after discharge within a care pathway defined by the hospital.

CLINICAL BOUNDARY

Different jobs. One accountability rule.

Each agent has a bounded role. The system can collect, structure, flag or draft; the clinician remains responsible for interpretation, change and sign-off.

01

Collect

AI Pre-Consultation organizes symptoms and history before the visit. It does not silently turn intake into a diagnosis.

02

Analyze

CDSA and HeartEngine return evidence-linked analysis, risks and options. The clinician decides what to accept or reject.

03

Draft & check

Record agents create an editable draft or a quality flag. The clinician reviews, edits and signs the final record.

04

Continue

AI Health Assistant follows a hospital-defined pathway, with escalation and human review kept visible.

03 / DESIGNED TO WORK WITH EXISTING SYSTEMS

Your hospital systems remain the source of truth.

Runda is designed to add a governed clinical intelligence layer to established digital workflows—not replace the systems hospitals already rely on. The integration approach is adapted to each institution's architecture, approved data boundaries and clinical priorities.

SYSTEM OF RECORD

HIS / EHR / EMR

Longitudinal clinical data and established workflows

CONTROLLED ACCESS

Approved integration

FHIR, HL7, APIs, interface services or governed data views

INTELLIGENCE FOUNDATION

Runda CDx

Normalization, knowledge and traceability

CLINICAL APPLICATION

Selected agent

A bounded task inside the care workflow, embedded or returned by API

ACCOUNTABILITY

Clinician review

Results return through an agreed workflow

POTENTIAL DATA DOMAINS

Encounters and history	Diagnoses and procedures
Medications and allergies	Laboratory results
Vital signs	Clinical notes
Diagnostic reports	Orders and care plans

DATA EXCHANGE

Use only the data the workflow needs.

Approved APIs can push or pull updates, and governed views can expose a bounded data set. Batch queries can be designed around update windows, patient IDs or report IDs.

WORKSTATION CONTEXT

Open the right agent with the right context.

A patient, visit, clinician, hospital and department context can be passed into an embedded module or an approved application route, so the result returns to the existing work pattern.

NETWORK & CONTROL

Keep the path observable.

Intranet reachability, HTTPS, access control, audit trails, backup and the connection between application and AI compute are part of the design—not afterthoughts.

Exact integration methods are confirmed with each organization based on its current systems, available interfaces, security requirements and enterprise architecture. The first design question is the clinical workflow—not a fixed technology stack.

04 / FROM DEMONSTRATION TO OPERATION

Start with understanding. Add infrastructure only when the use case is clear.

There are three different things people often call a "POC". Keeping them separate makes the next decision easier for IT, clinical teams and leadership.

01 Remote product demonstration

Pre-set sample cases show the workflow and interface. No patient data is connected, and no new AI server is required.

UNDERSTAND THE CAPABILITY

02 Focused validation with de-identified cases

The healthcare organization selects one agent, one clinical context and a small approved case set. Both sides agree the measures before reviewing the output.

TEST LOCAL USEFULNESS

03 On-site POC or production deployment

After clinical value and scope are agreed, deploy a dedicated environment, connect approved systems, complete security review and operate with audit and support.

RUN UNDER LOCAL GOVERNANCE

INFRASTRUCTURE FOLLOWS SCOPE

There is no honest server answer before the workflow is chosen.

Application & data services

Run the governed product layer and approved clinical context.

AI compute

Size model, accelerator, concurrency and latency after the agent and users are known.

Network & security

Design data transfer, result return, HTTPS, access boundaries and failure handling together.

Operations

Include monitoring, backup, release control and clinical acceptance from the beginning.

WHAT A FOCUSED PROOF SHOULD MEASURE

“It runs” is only the first gate.

Clinical usefulness

Does it help the intended decision or documentation task?

Safety & traceability

Can a clinician inspect the source, rule or evidence behind an output?

Workflow fit

Does it reduce work without adding a second disconnected system?

Adoption & reliability

Will users trust it, and does the integration behave consistently?

Runda's deployment plan provides a reference sequence of scope, foundation deployment, integration, testing, clinical fine-tuning and go-live. Timing depends on the selected agent, system access, security review and infrastructure readiness.

05 / BEYOND THE CATALOGUE

A clinical need does not have to fit an existing agent.

Runda works with clinical and IT teams to design, validate and deploy institution-specific agents around real clinical priorities.

01**Define the clinical need**

Users, decisions and current workflow

02**Map the system context**

Approved data, integration and access

03**Build the agent**

Local terminology, knowledge and workflow logic

04**Validate clinically**

Safety, usefulness and acceptance criteria

05**Deploy under local governance**

Controlled users, audit and measured operation

06 / AI SOVEREIGNTY

Healthcare organizations stay in control.

Runda provides the clinical AI foundation and agent-development capability. Each organization retains authority over its institutional data, operating boundaries and organization-specific AI assets created for its environment.

01

Data

The organization determines which clinical data an agent can access and for what purpose.

02

Environment

Deployment, residency and access controls follow locally approved boundaries.

03

Governance

The organization defines users, permissions, review requirements and acceptable use.

04

Local assets

Institution-specific agents, workflow configurations and local knowledge assets remain locally governed.

Ownership and usage rights for institution-specific assets are reflected in final commercial and IP terms. Runda's pre-existing platform technologies remain Runda intellectual property.

07 / SELECTED EVIDENCE

Operating experience across scale, specialty depth and localization.

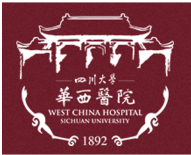
Four deployments show different dimensions of Runda's clinical AI experience. Each result belongs to its specific implementation context.



LOCALIZATION AT NETWORK SCALE

Mexico / Mexican Social Security Institute (IMSS)

78M	people served
1,700+	facilities
~15 sec	to analyze a two-year record



SCALE AND OPERATIONAL VALUE

China / West China Hospital

4,900	beds
10,000	records per day
15,584	reported clinician hours saved in one week



HIGH-RISK SPECIALTY DEPTH

China / Qilu Hospital of Shandong University

97%	reported accuracy
99.8%	reported compliance
0	reported missed high-risk cases



CONNECTED CARE CONTINUUM

China / TEDA International Cardiovascular Hospital

1 sec

reported generation time

2.7%

reported modification rate

~223

reported hours saved per week

Reported outcomes are specific to the cited deployments and are not a guarantee of results in another organization.



Clinical AI built on governed medical data.

Clinical AI Overview · 2026